

Lawrence County **Community Health Improvement Plan**

2024 - 2027

July 31, 2024



Public Health
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Lawrence County Health Department

OHIO
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Voinovich School of
Leadership and Public Service



LAWRENCE COUNTY HEALTH DEPARTMENT

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Public Health
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Lawrence County Health Department

Tony Virgin, MD
Health Commissioner

Jim Meadows, MD
Medical Director

LETTER FROM THE HEALTH COMMISSIONER

In keeping with Lawrence County Health Department's (LCHD) commitment to improve the health of our community, I am pleased to present our Community Health Improvement Plan.

In July 2022, LCHD participated in a Community Health Needs Assessment (CHNA), led by UK King's Daughters Medical Center. We conducted SWOT (strengths, weaknesses, opportunities, and threats) analyses with community partners and agencies in early 2023. A combination of the CHNA and SWOT analyses contributed to Lawrence County Health Department's Community Health Assessment. Adult mental health and addiction, youth mental health and addiction, and adult obesity were identified as major health concerns.

Under the guidance of the Ohio University's Planning, Evaluation, Education and Research (PEER) Team, and with participation from community agencies and partners, the Lawrence County Community Health Improvement Plan (CHIP) was developed. Community partners were assigned to one of three workgroups for the identified health priorities of adult mental health and addiction, youth mental health and addiction, and adult obesity. Strategies, goals, objectives, opportunities for policy change, and timelines were established. Implementation of our CHIP will begin August 1, 2024 and will be reviewed at least annually through August 1, 2027.

I would encourage residents of Lawrence County to read the CHIP and to share with us any concerns or suggestions you may have for improving the health of our community.

We are very grateful for our dedicated staff and community partners who worked together to develop our Community Health Improvement Plan, and I'd like to extend my sincere thanks to Dr. Tammy Kahrig and Katarina Kroutel for moving us through the process.

Sincerely,

Tony Virgin, MD
Health Commissioner

TABLE OF CONTENTS

INTRODUCTION	1
Community Health Improvement Plan Partners.....	2
Lawrence County Health Department Staff.....	3
Alignment with State and National Standards	4
Vision	4
OVERVIEW OF THE CHIP PROCESS	6
COMMUNITY HEALTH NEEDS ASSESSMENT DATA	8
Place and Health	8
Demographic Profile.....	10
Adult Health Data Summary	10
Youth Health Data Summary.....	13
PRIORITIZATION OF TOP HEALTH ISSUES.....	18
LAWRENCE COUNTY ASSETS.....	19
ACTION PLANS	20
MONITORING AND EVALUATING THE CHIP	24
BIBLIOGRAPHY.....	25
APPENDIX: CHIP PARTNERS' PRIORITIZATION OF TOP HEALTH ISSUES	27



Lawrence County Seal



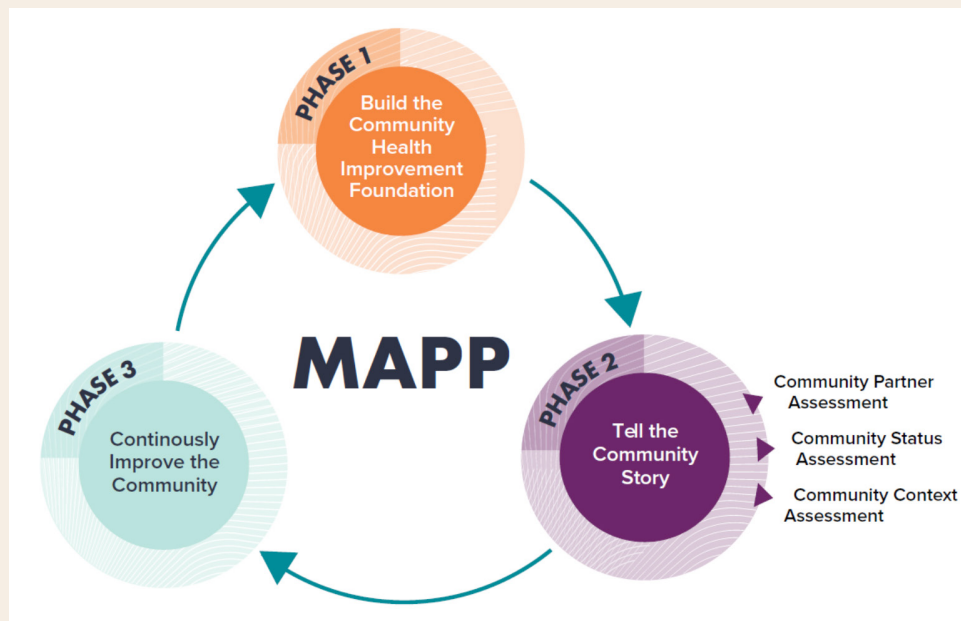
Blast Furnaces Historical Marker

INTRODUCTION

Lawrence County, Ohio, is located in the southernmost point of Ohio, adjacent to the Ohio River bordering West Virginia and Kentucky. Lawrence County Health Department (LCHD) is located in the county's largest city, Ironton, and serves all 14 townships in the county.

In 2022, LCHD began a community-driven strategic planning process to promote health for all in the county. Following the National Association of County & City Health Officials' model, Mobilizing for Action through Planning and Partnerships (MAPP 2.0)¹ depicted in Figure 1 below, LCHD collaborated with King's Daughters Medical Center and members of the community to conduct an assessment of the health of the county's residents in 2022-2023, including a community status survey, analysis of existing health data, and an analysis of the community context. Following the development of that Community Health Needs Assessment (CHNA), LCHD enlisted professionals from Ohio University's Voinovich School of Leadership and Public Service to facilitate the next phase of the process (Phase 3) aimed at addressing the health needs identified in the CHNA. Over a five-month period in 2024, LCHD and Ohio University staff engaged a diverse group of community organizations and members of the community (see list on page 2) in a series of discussions to review the health status of the community, prioritize needs, and determine strategies to improve the health of the community over the next three years. This work culminated in the Lawrence County Community Health Improvement Plan for 2024-2027, which is detailed in this report.

Figure 1. Mobilizing for Action through Planning and Partnerships (MAPP 2.0) Process



¹ National Association of City & County Health Officials, "Mobilizing for Action through Planning and Partnerships (MAPP) - NACCHO," Naccho.org, 2015, <https://www.naccho.org/programs/public-health-infrastructure/performance-improvement/community-health-assessment/mapp>.

Community Health Improvement Plan Partners

ADAMHS Board

Sue Shultz • Michele Bower

Anthem Blue Cross and Blue Shield (Elevance Health)

Laura Kuhn

Appalachian Family and Children First Council

Karen Reed

Area Agency on Aging

Jackie Corn • Connie Montgomery

Community-at-Large

Linda Holmes

Early Childhood Academy

Peggy Fuller

Faith-Based Community

Edward Holmes, Sr. • Rick Sturgill

Impact Prevention

Mollie Stevens • Eddie Neel

Mykel Delong

Ironton City School District

Joe Geletka

Ironton Lawrence County CAO Family Medical Centers

Nancy Lewis • Angela Pleasant

Jobs and Family Services

Jamie Murphy • Missy Evans

Lawrence County Board of Health

Brenda Shipley (retired school nurse)

Lawrence County Chamber of Commerce/ Economic Development

Shirley Dyer

Lawrence County Commissioners

Katrina Keith (also part of the faith-based community)

Lawrence County Coroner's Office

Drew Artis

Lawrence County Developmental Disabilities

Julie Monroe

Lawrence County Emergency Management

Mike Boster

Lawrence County Emergency Medical Services

Mac Yates

Lawrence County Juvenile Center

Brett Looney

Lawrence County Sheriff's Department

Mike Gore

Mended Reeds Mental Health

Kevin Lambert

Necco Center

Ruthanne Delong

Ohio University Southern Campus

Debbie Marinski, Dean

OSU Extension

Debbie Carpenter • Zoie Clay

Pathways

Scott Murphy

UK Kings Daughters Medical Center

Diva Justice • Kimberly Osborne

WIC

Suzie Cornwell

LCHD Staff

Debbie Fisher, • Dr. Jennifer Richards, Health
Administrator Planner/Accreditation
Coordinator

Angela Doyle, • Mohammad Abdulrahman,
Director of Nursing Epidemiologist

Paola Litton, • Phyllis Becker,
Health Educator Accounts Payable

Cindy Quillen, Emergency • Zach Schweinsberg,
Response Coordinator Fiscal Officer

Garrett Carpenter, • Camryn Zornes,
Registered Environmental Health Educator
Health Specialist

Lawrence County Health Department Staff



LCHD Leadership Team (L-R) Zach Schweinsberg, Fiscal Officer; Angela Bostick-Doyle, Director of Nursing; Tony Virgin, MD, Health Commissioner; Debbie Fisher, Administrator; Paul O'Banion, Environmental Health Director



Nursing Staff Front Row (L-R) Brenda Schweinsberg, Public Health Nurse; Georgia Dillon, Women's Health Nurse Practitioner; Angela Bostick-Doyle, Director of Nursing; Sandy Newman, Public Health Nurse

Back Row (L-R) Sherry Prince, Public Health Nurse/Insurance Navigator; Stephanie Barnett, Immunization Nurse; Debbie Fisher, Administrator



Fiscal Staff (L-R) Phyllis Becker, Accounts Payable; Zach Schweinsberg, Fiscal Officer



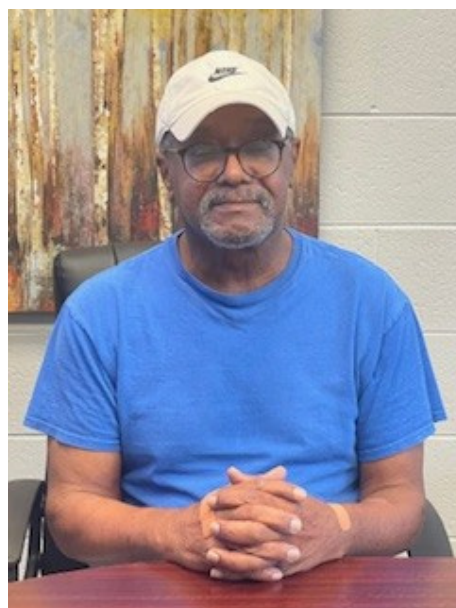
Front office staff (L-R) Katie Manis, Clerk; Frannie Klaiber, Biller; Paula Carpenter, Vital Statistics Registrar; Linda Butler, Receptionist



Emerging Projects Staff (L-R) Mohammad Abdulrahman, Epidemiologist; Cindy Quillen, Emergency Response Coordinator; Paola Litton, Health Educator; Jennifer Richards, Health Planner/Accreditation Coordinator



Environmental Health Staff (L-R) Katie Manis, Clerk; Steve Rudd, REHS; Garrett Carpenter, REHS; Mitchell Jenkins, Nuisance Officer; Jessica Stapleton, Clerk



Dennis Carter (custodian)

Alignment with State and National Standards

Ohio Revised Code² requires that local health departments carry out the 10 Essential Public Health Services and the Foundational Capabilities set by the Public Health Accreditation Board (PHAB). The PHAB standards for accreditation require that health departments assess and monitor population health status, factors that influence health, and community needs and assets (Domain 1) and that they develop and implement community health improvement strategies collaboratively (Domain 5).³ Ohio Revised Code specifies that health departments (and hospitals) must complete a community health assessment followed by a community health improvement plan every three years.⁴ Lawrence County Health Department adhered to the PHAB standards in the development of the Community Health Needs Assessment (CHNA) and the Community Health Improvement Plan (CHIP), in accordance with Ohio Revised Code.

Throughout the Phase 3 discussions of the CHIP process, the Community Health Improvement Plan (CHIP) Partners were engaged in examining health improvement plans developed at both the state and national levels. The CHIP Partners reviewed materials from the United States Department of Health and Human Services' Healthy People 2030,⁵ which identifies public health priorities to help individuals, organizations, and communities across the U.S. improve health and well-being. The CHIP Partners also examined the Ohio Department of Health's State Health Improvement Plan (SHIP),⁶ which is a tool to strengthen state and local efforts to improve health, well-being, and economic vitality in Ohio and address the complex health challenges identified in the 2019 State Health Assessment. The CHIP Partners followed the framework outlined in Ohio's SHIP shown in Figure 2 when identifying priority factors, priority health outcomes, and evidence-based strategies to include in the CHIP. The CHIP's alignment with the priorities and indicators of Ohio's SHIP and Healthy People 2030 is detailed on each action plan beginning on page 20.

Vision

After reviewing materials from Healthy People 2030 and Ohio's SHIP, the CHIP Partners developed the following vision for Lawrence County:

A society in which all people can achieve their full potential for health and well-being across the lifespan through making the healthier choice the easier choice and overcoming barriers to economic vitality.

² "Section 3709.22 - Ohio Revised Code | Ohio Laws," n.d., Codes.ohio.gov, accessed July 18, 2024, <https://codes.ohio.gov/ohio-revised-code/section-3709.22>.

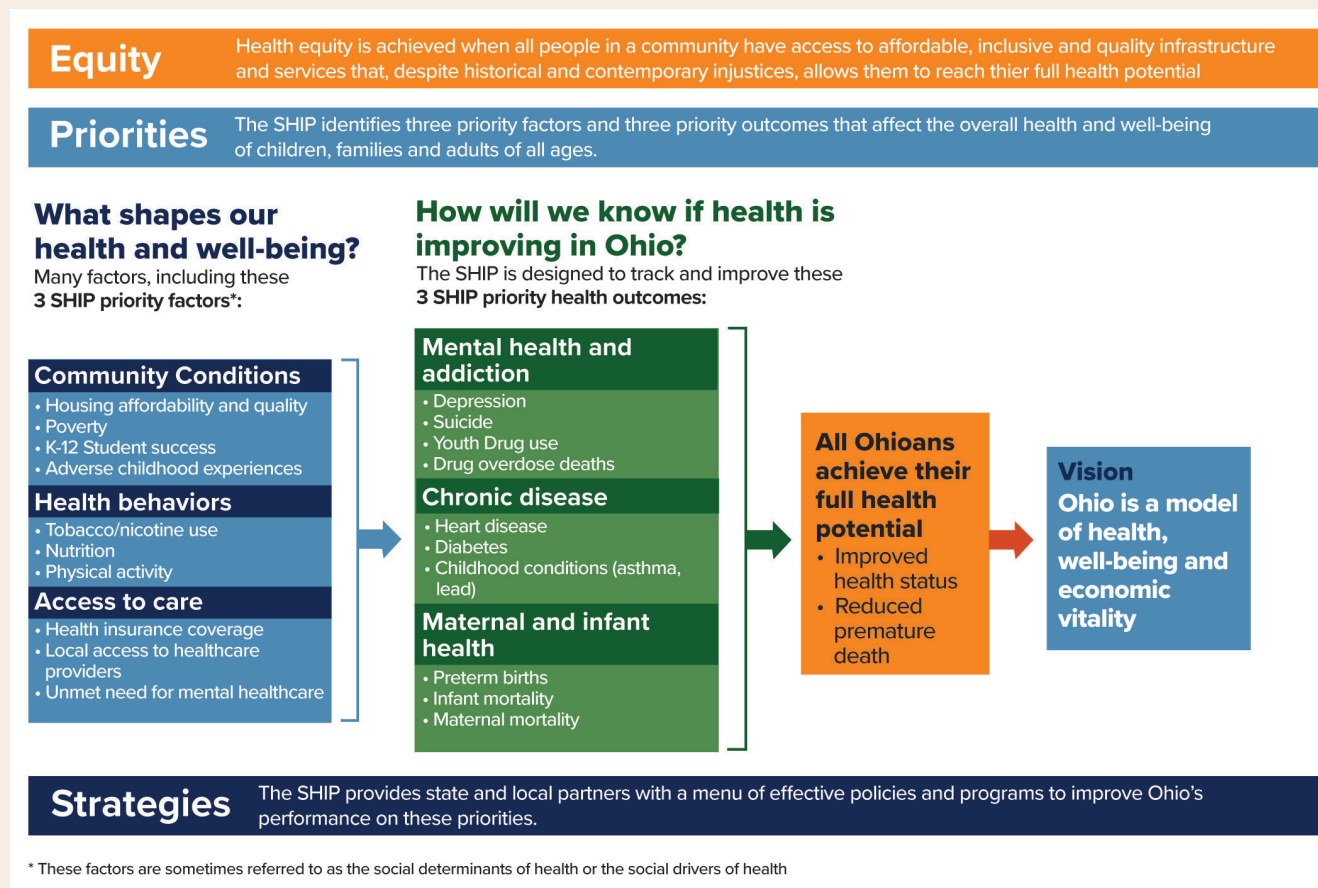
³ Public Health Accreditation Board, "Standards & Measures for Initial Accreditation," n.d., <https://phaboard.org/wp-content/uploads/Standards-Measures-Initial-Accreditation-Version-2022.pdf>.

⁴ "Section 3701.981 - Ohio Revised Code | Ohio Laws," n.d., Codes.ohio.gov, accessed July 18, 2024, <https://codes.ohio.gov/ohio-revised-code/section-3701.981>.

⁵ U.S. Department of Health and Human Services, "Healthy People 2030," Healthy People 2030, Office of Disease Prevention and Health Promotion, 2020, <https://health.gov/healthypeople>.

⁶ Ohio Department of Health, "State Health Improvement Plan," 2020, <https://dam.assets.ohio.gov/image/upload/odh.ohio.gov/SHIP/2020-2022/2020-2022-SHIP.pdf>.

Figure 2. Ohio's State Health Improvement Plan Framework



Ironton Riverfront

OVERVIEW OF THE CHIP PROCESS

Over a five-month period in 2024, the CHIP Partners met to develop the Lawrence County CHIP. Below is a description of the process and resources that were utilized in accordance with MAPP 2.0:

A. Met to Review Data and Prioritize Issues.

- a. Discussed Lawrence County assets.
- b. Reviewed health status data from the CHNA survey and context assessment as well as data on health behaviors and outcomes for adults and youth compared to the state and the nation where available.
- c. Discussed data and began to prioritize issues considering:
 - i. Relevance of the issue to community members;
 - ii. Magnitude/severity of the issue;
 - iii. Urgency to solve the issue;
 - iv. Impact of the issue on community members impacted by inequities;
 - v. Trending health concerns;
 - vi. Availability of resources to address the issue;
 - vii. Opportunity to apply upstream strategies to address the issue;
 - viii. Societal, political, historical, and cultural context of the issue; and
 - ix. Impact of the issue on different census tracts in the county.
- d. Ranked each of the adult health issues present in the health status data.
- e. Ranked each of the youth health issues present in the youth health status data.

B. Shared Results of the Prioritization.

C. Set Up Priority Issue Groups.

D. Completed Survey of Current Programs, Services, and Interventions Related to the Top Three Priority Issues Identified.

E. Met in Small Groups to Identify Root Causes and Power Analysis for Each of the Three Issues.

- a. Completed root cause analysis by using either the Fishbone Diagram or the 5 Whys tool for each issue.
- b. Completed a power analysis of each issue to identify potential partners and opponents.

F. Developed Issue Profiles for Each of the Three Issues.

G. Met to Develop Vision, Shared Goals, and Long-Term Measures for Each Priority Issue.

- a. Reviewed and discussed research related to the three priority issues, especially regarding health disparities among populations and the impact of adverse childhood experiences.
- b. Discussed Healthy People 2030 and Ohio's State Health Improvement Plan.
- c. Created shared vision for Lawrence County.
- d. Met in Priority Issue Groups to develop shared goals and measures for collective impact.

H. Reviewed Evidence-Based Strategies from State and National Resources.

- I. Met in Priority Issue Groups to Select Evidence-Based Strategies and Develop Action Plans for Each.**
- a. Reviewed completed Goal Development Worksheets from previous meeting.
 - b. Reviewed and discussed evidence-based strategies.
 - c. Brainstormed strategies.
 - d. Evaluated possible strategies considering the PEARL Test.⁷
 - e. Developed action plans for priority health issue, including the objective/strategy, priority populations, measures, action steps, lead contact and partners, and timeframe.
- J. Shared the Action Plans and Solicited Feedback from the CHIP Partners.**

⁷ National Association of County & City Health Officials, “Guide to Prioritization Techniques,” n.d., <https://www.naccho.org/uploads/downloadable-resources/Guide-to-Prioritization-Techniques.pdf>.

COMMUNITY HEALTH NEEDS ASSESSMENT DATA

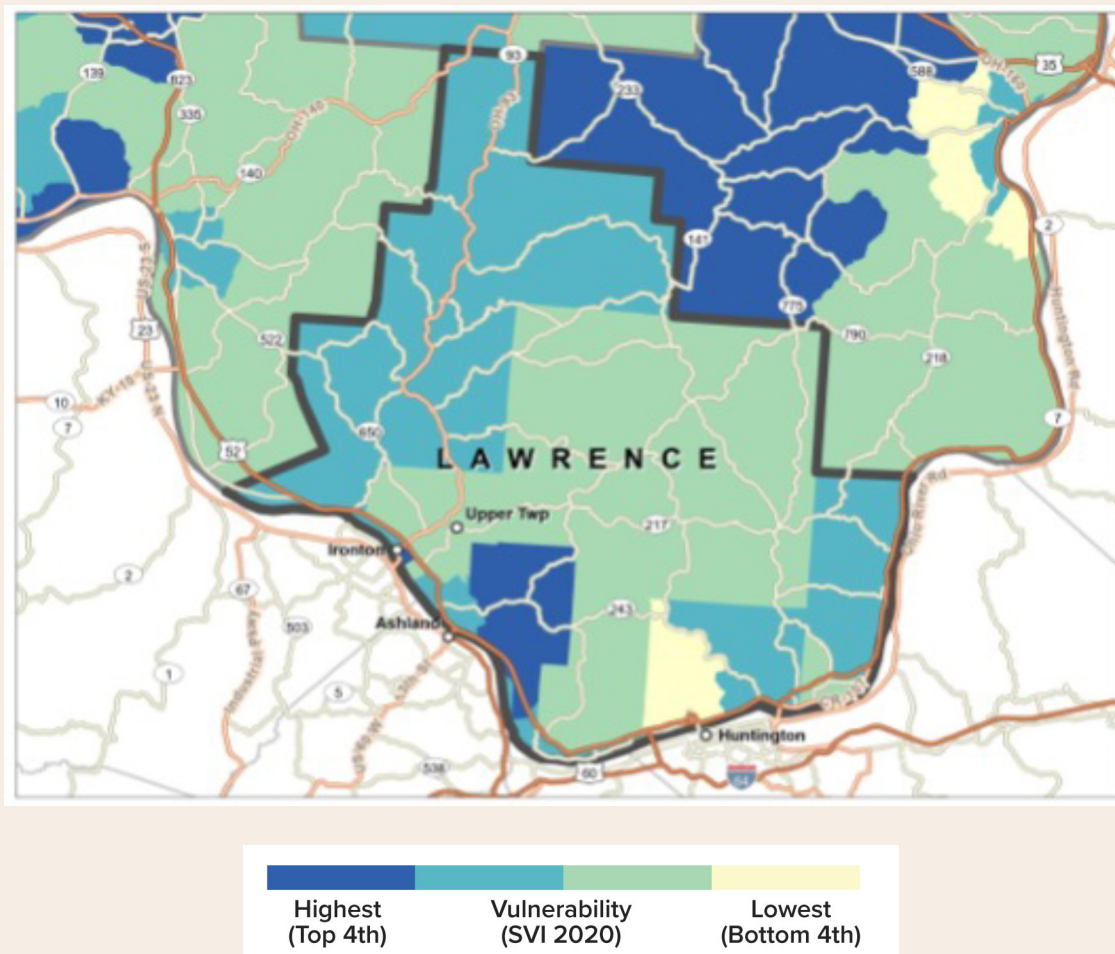
Place and Health

The Social Vulnerability Index (SVI) is a score that compares communities by the level of impact they are likely to experience from natural or human-caused disasters, or disease outbreaks. The scores include **socioeconomic status, household composition and disability, race/ethnicity and language, and housing and transportation**. Higher scores mean greater risk.

The map of Lawrence County below shows the Overall Social Vulnerability scores in each census tract in the county.

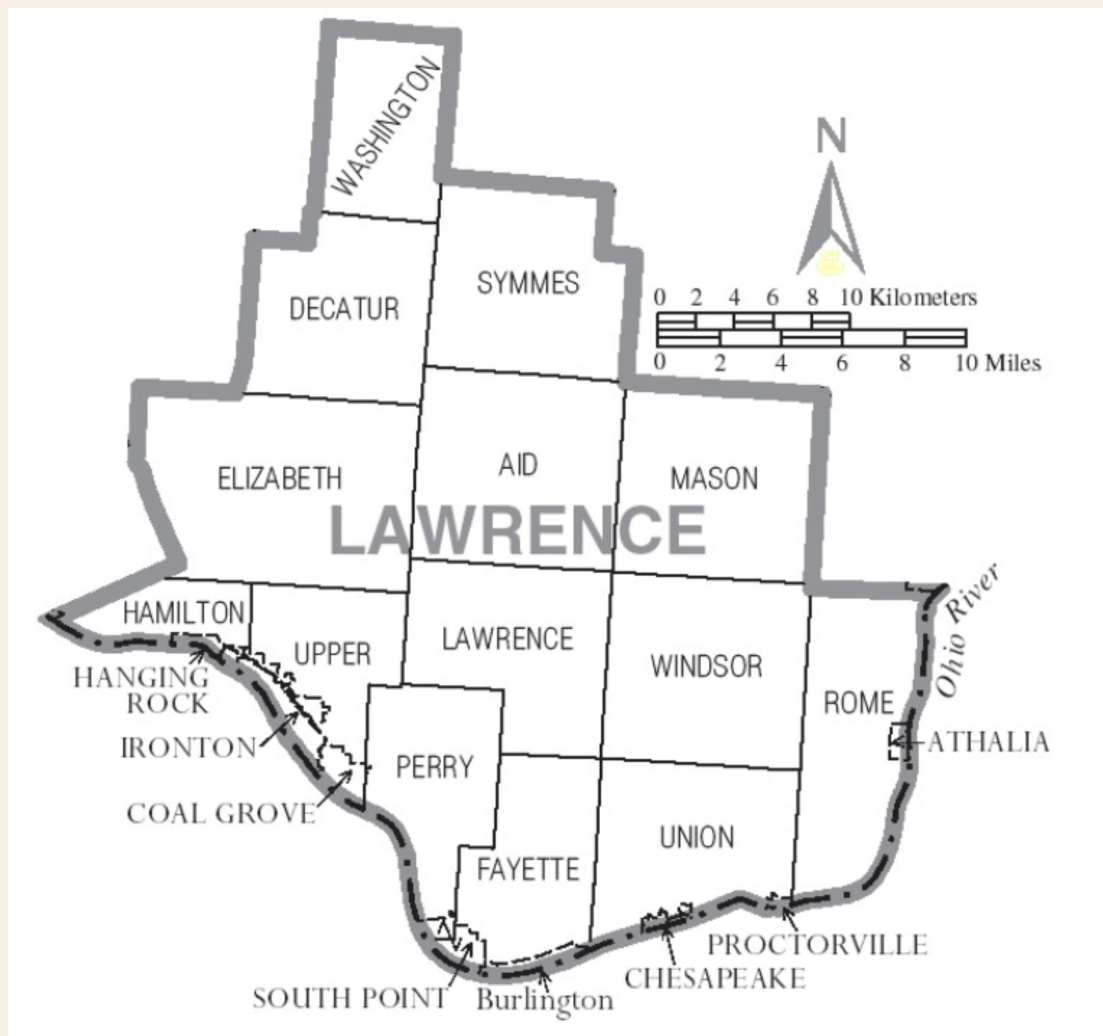
In this map, the dark blue parts of Lawrence County have the higher levels of social vulnerability. These areas are at greater risk for negative impacts from natural disasters or outbreaks of disease.

Figure 3. Lawrence County Social Vulnerability Index⁸



⁸ CDC/ATSDR, "Social Vulnerability Index 2020 Lawrence County, Ohio," accessed February 13, 2024, <https://svi.cdc.gov/map/>.

Figure 4. Lawrence County, Ohio, Townships⁹



A comparison of the Social Vulnerability Index with the townships shown in Figure 4 indicates that the most vulnerable part of the county is Perry Township. Coal Grove, South Point, Washington, Decatur, Symmes, Hamilton, all but the southernmost part of Rome, and the eastern part of Union townships are also highly vulnerable communities according to the SVI.

⁹ Paola Litton, Jennifer Richards, and Mohammad Abdulrahman, "Community Health Needs Assessment 2023 Lawrence County, Ohio," accessed February 27, 2024, <https://www.lawcohd.org/community-health-assessment>.

Demographic Profile

Table 1: Residents' Age, Disabilities, Residence, and Race/Ethnicity Compared to the State and the Nation¹⁰

	Lawrence County	Ohio	U.S.
AGE, RESIDENCE, AND DISABILITY STATUS			
Below 18 Years of Age	21.7%	22.1%	22.2%
65 and Older	19.3%	17.8%	16.8%
Rural	45.9%	22.1%	19.3%
Persons with Disabilities ¹¹	23.3%	13.9%	13.5%
RACE AND ETHNICITY			
White	94.4%	77.7%	61.6%
Non-Hispanic Black	2.1%	12.8%	12.4%
Hispanic	1.0%	4.3%	18.7%
Asian	0.5%	2.7%	6.0%
American Indian or Alaska Native	0.3%	0.3%	1.1%
Native Hawaiian or Other Pacific Islander	0%	0.1%	0.2%
% Not Proficient in English	0%	1%	0.3%

Adult Health Data Summary

Table 2: Overall Health Outcomes and Mental Health of Residents Compared to the State and the Nation¹²

	Lawrence County	Ohio	U.S.
OVERALL HEALTH OUTCOMES			
Premature Death: Years of Potential Life Lost before Age 75 per 100,000	11,900	8,700	7,300
Life Expectancy	72.7	76.5	78.5
MENTAL HEALTH			
Suicide Death Rate per 100,000 ¹³	21.9	14.8 ¹⁴	14.1
Frequent Mental Distress	19%	16%	14%
Poor Mental Health Days	5.7	5.0	4.4
Mental Health Providers (1 provider per number of people)	200:1	330:1	340:1

¹⁰ County Health Rankings & Roadmaps, "Lawrence County, Ohio," accessed February 11, 2024, <https://www.countyhealthrankings.org/explore-health-rankings/ohio/lawrence?year=2023>.

¹¹ Shreya Paul, Shannon Rogers, Stacia Bach, and Andrew Houtenville, "2023 State Report for Ohio County-Level Data: Prevalence," University of New Hampshire, Institute on Disability, 2023, <https://disabilitycompendium.org/compendium/2023-state-report-for-county-level-data-prevalence/OH>.

¹² County Health Rankings & Roadmaps, "Lawrence County, Ohio."

¹³ Ohio Department of Health, "Suicide Demographics and Trends, Ohio, 2021," accessed February 27, 2024, <https://odh.ohio.gov/know-our-programs/violence-injury-prevention-program/media/2021-ohio-suicide-report>.

¹⁴ Centers for Disease Control and Prevention, "Suicide Data and Statistics," accessed February 27, 2024, <https://www.cdc.gov/suicide/suicide-data-statistics.html>.

Table 3: Overall Health Outcomes and Mental Health of Residents Compared to the State and the Nation¹⁵

	Lawrence County	Ohio	U.S.
PHYSICAL ACTIVITY, NUTRITION, AND WEIGHT STATUS			
Overweight or Obese	40%	36%	32%
Physical inactivity	33%	24%	22%
Food Insecurity	18%	12%	12%
Consumed Fruit Less than 1 Time per Day ¹⁶	50% (Region)	42.7%	39.3%
Consumed Vegetables Less than 1 Time per Day ¹⁷	21.2% (Region)	20.2%	21.2%
Access to exercise opportunities	57%	84%	84%
Limited Access to Healthy Foods	4%	7%	6%
ALCOHOL CONSUMPTION			
Excessive Drinking	17%	19%	19%
Alcohol-Impaired Driving Deaths	13%	33%	27%
DRUG USE			
Drug Overdose Deaths	52	38	23
Drug Overdose Rate per 100,000 2020-2022 ¹⁸	89.2	43.6	32.4 ¹⁹
TOBACCO USE			
Current Smoker	26%	20%	16%
Current Electronic Vaping User, All Adults ²⁰	6.11% ²¹ (Region)	5.9%	4.5%
Current Electronic Vaping User, Ages 18-24	-----	24.1% ²²	11.0% ²³
HEALTH CARE ACCESS AND UTILIZATION			
Primary Care Physicians	2,270:1	1,290:1	1,310:1
Dentists	3,020:1	1,550:1	1,310:1
Preventable Hospital Stays	4,822	3,278	2,809
Mammography Screening	30%	40%	37%
Flu Vaccinations	46%	53%	51%

¹⁵ County Health Rankings & Roadmaps, "Lawrence County, Ohio."

¹⁶ Ohio Department of Health, "Behavioral Risk Factor Surveillance System: 2019 Annual Report," accessed February 27, 2024, <https://odh.ohio.gov/know-our-programs/chronic-disease/data-publications/ohio-2019-brfss-annual-report>.

¹⁷ Ohio Department of Health, "Behavioral Risk Factor Surveillance System."

¹⁸ Ohio Department of Health, "2022 Ohio Unintentional Drug Overdose Death Report," accessed February 27, 2024, <https://odh.ohio.gov/know-our-programs/violence-injury-prevention-program/media/2022-ohio-drug-overdose-report>.

¹⁹ Merianne R. Spencer, Arialdi M. Miniño, and Margaret Warner, "Drug Overdose Deaths in the United States, 2001–2021," NCHS Data Brief no. 457 (December 2022): 1–8, National Center for Health Statistics, <https://dx.doi.org/10.15620/cdc:122556>.

²⁰ Kevin Lombardi, "Study: Vaping Statistics by State," last modified September 5, 2023, <https://www.drugwatch.com/e-cigarettes/vape-stat-by-state/>.

²¹ Centers for Disease Control and Prevention, "Behavioral Risk Factors: Selected Metropolitan Area Risk Trends MMSA Prevalence Data (2011 to Present)," accessed March 12, 2024, https://data.cdc.gov/Behavioral-Risk-Factors/Behavioral-Risk-Factors-Selected-Metropolitan-Area/j32a-sa6u/data_preview.

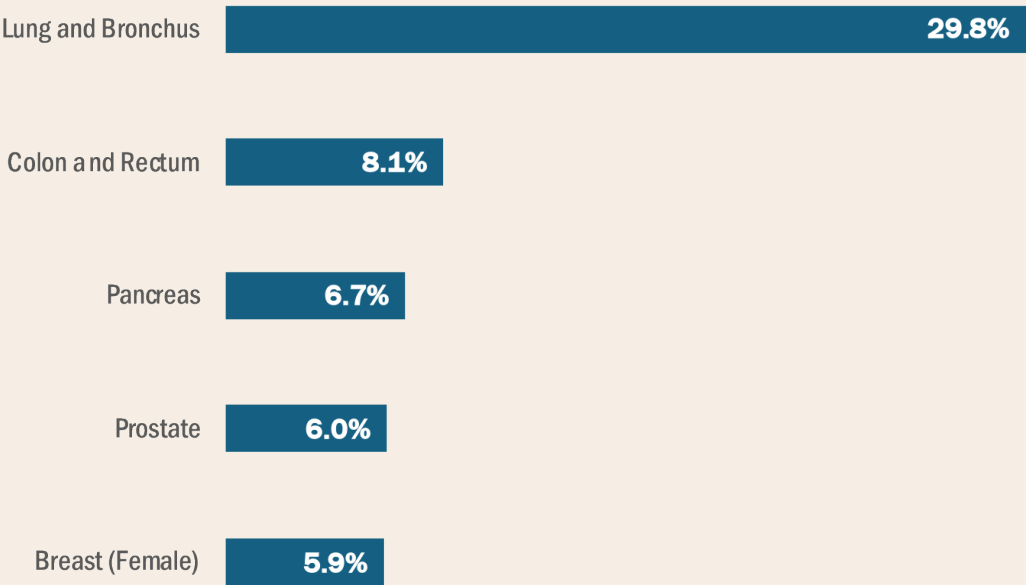
²² Ohio Department of Health, "Tobacco Use Among Adults in Ohio, 2021," accessed March 12, 2024, <https://odh.ohio.gov/know-our-programs/tobacco-use-prevention-and-cessation/data>.

²³ Ellen A. Kramarow and Nazik Elgaddal, "Current Electronic Cigarette Use Among Adults Aged 18 and Over: United States, 2021," NCHS Data Brief no. 475 (July 2023), <https://www.cdc.gov/nchs/data/databriefs/db475.pdf>.

Table 4: Overall Health Outcomes and Mental Health of Residents Compared to the State and the Nation²⁴

	Lawrence County	Ohio	U.S.
CANCER INCIDENCE AND MORTALITY PER 100,000			
Cancer Incidence Rate	553.5	465.3	438.7
Cancer Death Rate	217.8	166.1	149.4
Cancer Death Rate-White	220.1	165.4	150.5
Cancer Death Rate-Black	175.1	183.3	169.6
Cancer Death Rate-Men	272.6	199.6	177.5
Cancer Death Rate-Women	175.5	142.0	128.7

Figure 5. Top Five Cancers by Percentage of Cancer Deaths in Lawrence County, 2016–2020²⁵



²⁴ Ohio Department of Health, “Lawrence County Cancer Profile 2023,” accessed February 12, 2024, <https://odh.ohio.gov/know-our-programs/ohio-cancer-incidence-surveillance-system/countyprofiles/lawrence-county>.

²⁵ Ohio Department of Health, “Lawrence County Cancer Profile 2023.”

Table 5. Chronic Disease Incidence and Mortality of Residents Compared to the State and the Nation²⁶

	Lawrence County	Ohio	U.S.
CHRONIC DISEASE INCIDENCE AND MORTALITY			
High Blood Pressure Prevalence 2019	40.8%	-----	48.1% ²⁷
Heart Disease Prevalence 2020	9.4%	-----	5.5% ²⁸
Heart Disease Death Rate per 100,000 ²⁹	227.9	192.3	164.9
Heart Disease Death Rate per 100,000-Black	336.6	229.7	216.3
Heart Disease Death Rate per 100,000-White	227.3	191.4	168.5
Heart Disease Death Rate per 100,000-Men	290.1	243.8	209.4
Heart Disease Death Rate per 100,000-Women	175.0	151.3	128.3
Diabetes Prevalence ³⁰	14.8%	12.2%	10.9%
Diabetes Death Rate per 100,000 ³¹	33.7	29.5	31.1 ³²

Youth Health Data Summary 2022–2023

Table 6. Mental Health of Youth Residents Compared to the State³³

	Lawrence County	Ohio
Mental Health		
Anxiety issues warranting further exploration by a mental health professional	33%	29%
Depression issues warranting further exploration by a mental health professional	25%	21%
Mental health issues warranting further exploration by a mental health professional	27%	23%
Felt sad or hopeless almost every day for two weeks or more in a row during the past year	41%	37%
Seriously considered attempting suicide during the past year	15%	14%

²⁶ Centers for Disease Control and Prevention, “Interactive Atlas of Heart Disease and Stroke 2018-2020,” accessed March 1, 2024, <https://nccd.cdc.gov/DHDSPAtlas/reports.aspx?state=GA&themeld=2>.

²⁷ Centers for Disease Control and Prevention, “Facts About Hypertension,” accessed March 1, 2024, <https://www.cdc.gov/bloodpressure/facts.htm>.

²⁸ Centers for Disease Control and Prevention, “Health, United States, 2020-2021,” accessed March 1, 2024, <https://www.cdc.gov/nchs/health-us/topics/heart-disease-prevalence.htm#featured-charts>.

²⁹ Centers for Disease Control and Prevention, “Interactive Atlas of Heart Disease and Stroke 2018-2020.”

³⁰ Ohio Department of Health, Ohio Department of Medicaid, Ohio Department of Administrative Services, and Ohio Commission on Minority Health, “Ohio Diabetes Action Plan 2021,” accessed February 20, 2024, <https://odh.ohio.gov/know-our-programs/chronic-disease/data-publications/ohio-diabetes-action-plan-2021>.

³¹ Ohio Department of Health, “Ohio Diabetes Action Plan.”

³² Centers for Disease Control and Prevention, “National Diabetes Statistics Report,” accessed February 27, 2024, <https://www.cdc.gov/diabetes/data/statistics-report/index.html>.

³³ Ohio Department of Mental Health & Addiction Services, Ohio Department of Health and Ohio Department of Education and Workforce, “Ohio Healthy Youth Environments Survey-OHYES! Report for Lawrence County 2022-2023,” (2023): 1–73, <https://youthsurveys.ohio.gov/reports-and-insights/ohyes-reports/2022-2023/ohyes-lawrence-county-report-2022-2023>.

Table 7. Health Behaviors and Health Care Utilization of County Youth Residents Compared to the State³⁴

	Lawrence County	Ohio
Physical Activity, Nutrition, and Weight Status		
Overweight or Obese	45%	37%
Zero days of physical activity	14%	12%
Drank no soda or pop in the last week	21%	29%
Consumed less than 1 serving of fruits and vegetables on average day	26%	21%
Alcohol Consumption		
Consumed alcohol on one or more occasions during the past 30 days	12%	9%
Binge Drinking: At least one day with 4/5 or more drinks of alcohol within a couple of hours during the past 30 days	53%	44%
Tobacco Use		
Used cigarettes, cigars, chewing tobacco, snuff, or dip on one or more of the past 30 days	6%	3%
Ever used electronic vapor product	24%	18%
Drug Use		
Taken any prescription drugs without a doctor's prescription during lifetime	7%	6%
Used marijuana or hashish during the past 30 days	5%	6%
Past year use of any illicit substances (cocaine, inhalants, heroin, methamphetamines, ecstasy, hallucinogenic drugs, steroids, and synthetic marijuana)	4%	3%
Teen Births		
Teen Births 2023 per 1,000 females age 15–19 ³⁵	34	21
Health Care Utilization		
Visited a doctor or a nurse for a physical exam when you were not sick or injured in the last 12 months	51%	55%
Visited a dentist for a check-up, exam, teeth cleaning, or other dental work in the last 12 months	63%	69%

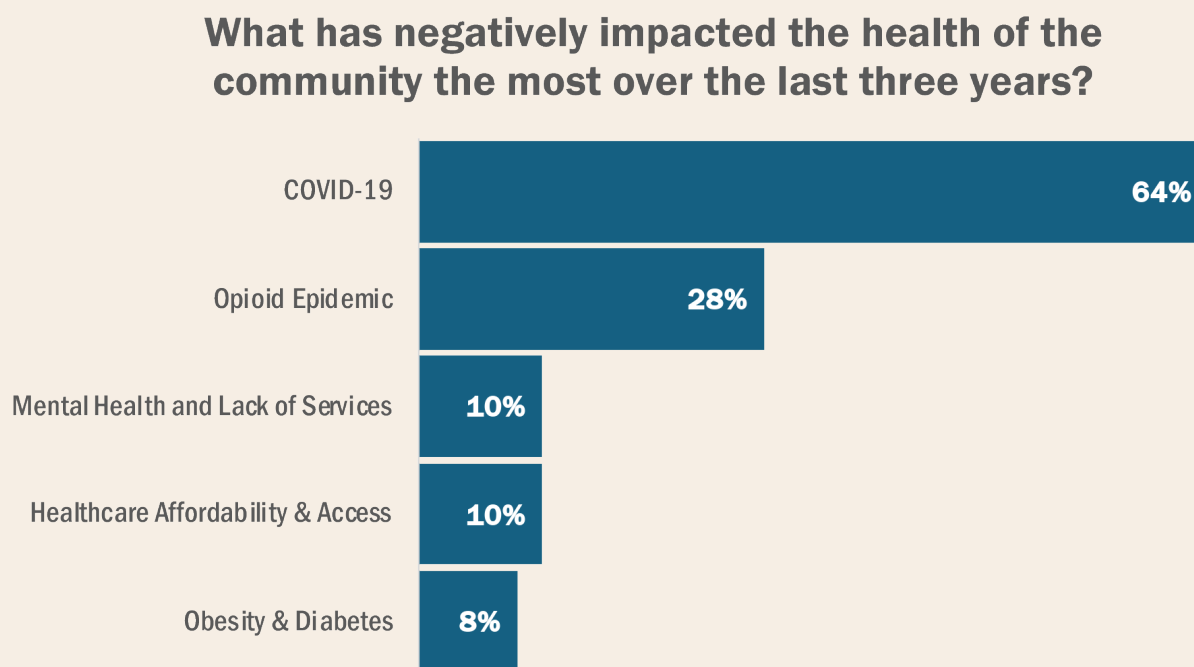
³⁴ Ohio Department of Mental Health & Addiction Services, "Ohio Healthy Youth Environments Survey-OHYES! Report."

³⁵ County Health Rankings & Roadmaps, "Lawrence County, Ohio."

Figure 6. Lawrence County's Top Five Weaknesses Identified by Stakeholders³⁶



Figure 7. Residents' Perceptions of Top Five Issues Negatively Impacting the Health of the Community³⁷

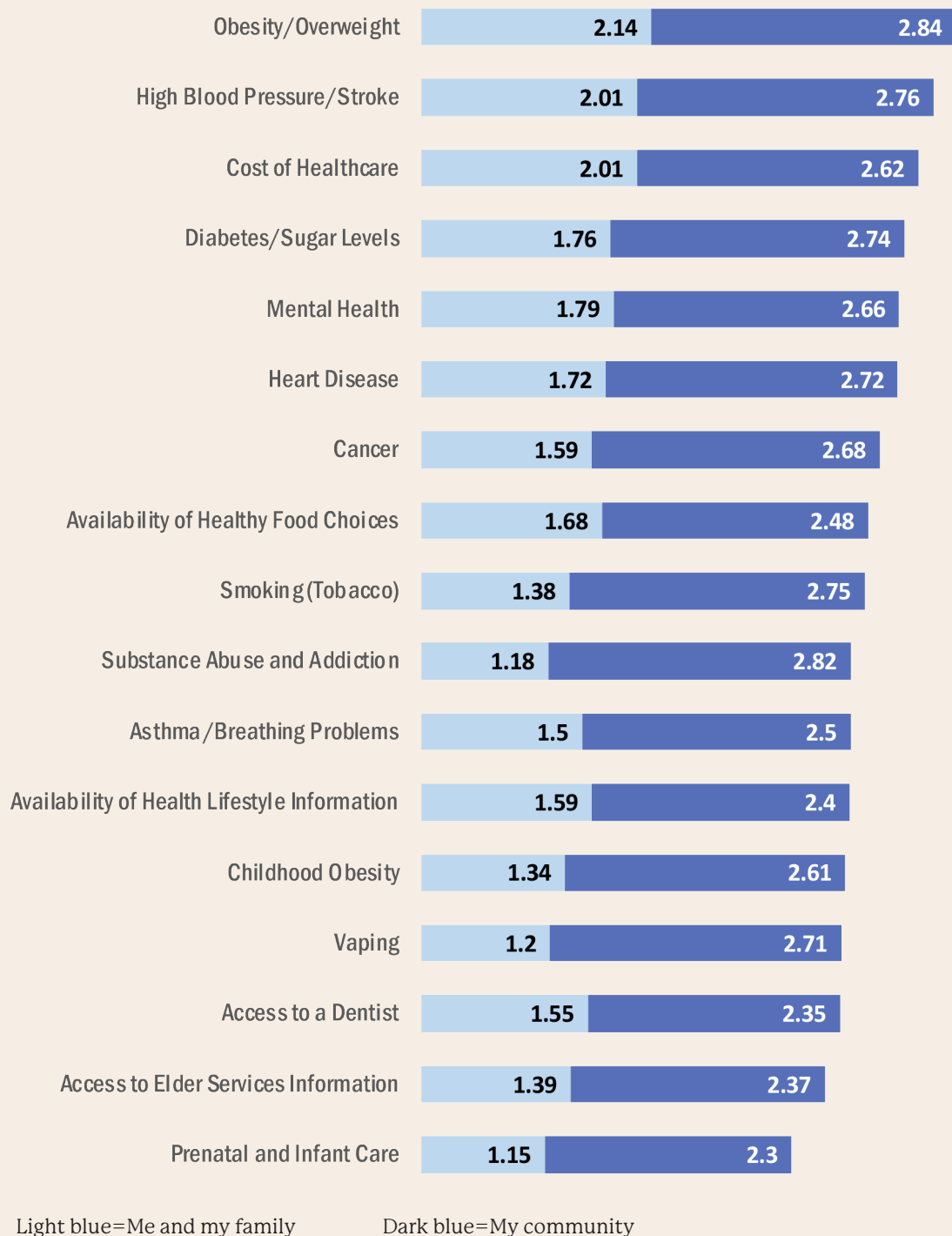


³⁶Paola Litton, Jennifer Richards, and Mohammad Abdulrahman, "Community Health Needs Assessment 2023 Lawrence County, Ohio."

³⁷King's Daughters Medical Center, "2022 Community Health Needs Assessment Survey, Lawrence County Data," provided by Debbie Fisher, Lawrence County Health Department.

Figure 8. Residents' Rankings of Health Issues Most Impacting their Health/Health of their Families and Health of the Community³⁸

How much do these health issues impact me and my family, and my community?



³⁸ King's Daughters, "2022 Community Health Needs Assessment Survey, Lawrence County Data."

Figure 9. Residents' Perceptions of Resources Needed in the Community³⁹



³⁹King's Daughters, "2022 Community Health Needs Assessment Survey, Lawrence County Data."

PRIORITIZATION OF TOP HEALTH ISSUES

Following a review and discussion of the health status data for adults and youth, the CHIP Partners were asked to consider the following criteria for each of the key areas:

- a) Relevance of the issue to community members
- b) Magnitude/severity of the issue
- c) Urgency to solve the issue;
- d) Impact of the issue on community members impacted by inequities;
- e) Trending health concerns;
- f) Availability of resources to address the issue;
- g) Opportunity to apply upstream strategies to address the issue;
- h) Societal, political, historical, and cultural context of the issue; and
- i) Impact of the issue on different census tracts in the county.

The CHIP Partners then completed a confidential ranking via polling software of the top health issues for both adults and youth. The votes were compiled by weighting each ranking of number one as three points, each ranking of number two as two points, and each ranking of number three as one point. See Appendix A for the results of the ranking. The top three issues that were prioritized for the CHIP (and restated to align with Ohio's SHIP) were:

- 1. Youth Mental Health and Addiction
- 2. Adult Mental Health and Addiction
- 3. Adult Obesity

LAWRENCE COUNTY ASSETS

During the CHIP prioritization, goal development, and action planning meetings, several key themes emerged as assets in Lawrence County: collaboration among organizations, a compassionate community, and emphasis on recreational activities. Residents of Lawrence County find the close-knit community and small-town atmosphere to be a great asset to the area. Many emphasize the continuity of working with familiar faces on various projects year after year, fostering strong connections. Throughout the pandemic, organizations strengthened their relationships, leading to a strong culture of collaboration across the community. The recreational offerings of Wayne National Forest, Lake Vesuvius, the 50 ponds in the county, and the Splash Waterpark are highly valued, enhancing the county's appeal. During crises, the deeply ingrained Appalachian mindset and strong family bonds underscore a shared sense of support and solidarity. The county's location within a tri-state area is seen as advantageous, providing additional resources and opportunities. Partnerships with institutions like Ohio University Southern further enhance the community, offering venues for events and valuable educational programs. Lastly, the historical heritage of the area adds depth and significance to Lawrence County.



Pig Iron Furnace Johns Creek



Tanks Memorial Stadium Marker

ACTION PLANS

PRIORITY 1: Youth Mental Health and Addiction

Mental health is one of the top issues impacting the health of Lawrence County youth. Forty-one percent of adolescents in grades 7–12 report feeling sad or hopeless almost every day for two weeks or more in a row during the past year, and 15 percent report they seriously considered attempting suicide during the past year. Research indicates that populations at high risk for poor mental health include: **females; lesbian, gay, and bisexual individuals; low-income individuals, and individuals with a disability**. Although females are more likely to experience depression, males are much more likely to attempt suicide. **Lesbian, gay, and bisexual students** are about four times more likely to have attempted suicide than their heterosexual peers. **Black students** are also more likely than their peers to have attempted suicide.

Substance use rates for Lawrence County youth are higher than the use rates for Ohio youth. Twenty-four percent of Lawrence County youth report having used an electronic vaping product, compared to 18 percent of Ohio youth. Seven percent of Lawrence County youth report having taken prescription drugs without a doctor's prescription, and approximately five percent report using marijuana in the last 30 days or using an illicit substance in the past year.

Adverse childhood experiences (ACEs) are known to be a significant contributing factor for poor mental health and substance use. Childhood trauma, including exposure to maternal SUD, predicts early onset of substance use and mental health issues in children. Those with six or more ACEs have significantly increased rates of depression and suicide. Persons with five or more ACEs have higher risks of early smoking initiation, current smoking, and heavy smoking. For males, physical abuse, parental divorce and economic problems are strong individual predictors for developing an illicit drug use disorder. Mental health trauma and substance abuse patterns are often transmitted across generations.

GOAL: Increase the number of schools allowing anti-vaping campaigns and expand opportunities for youth engagement.

ALIGNMENT WITH NATIONAL PRIORITIES: Healthy People 2030 Tobacco: Reduce E-Cigarette Use in Adolescents; Healthy People 2030: Improve Mental Health and Well-Being of Adolescents

ALIGNMENT WITH OHIO'S STATE HEALTH IMPROVEMENT PLAN (SHIP): Health Behavior: Reduce All Tobacco/Nicotine Use; Priority Health Outcome: Mental Health and Addiction

OBJECTIVE, STRATEGY	PRIORITY POPULATIONS	MEASURES	ACTION STEPS	LEAD CONTACT	TIMEFRAME	STATUS
1.1 Expand CATCH My Breath vaping education program to 80% of Lawrence County middle schools every year.	Youth grades 6–8	Aligned with SHIP HB2. Youth All Tobacco/Nicotine Use: Reduce the percentage of high school students who have used tobacco, e-cigarettes, or other vaping products in the past 30 days, especially among gay, lesbian, or bisexual students.	1. Present CATCH My Breath curriculum at superintendents meeting. 2. Arrange meetings with principals, school nurses and guidance counselors to explore implementing CATCH My Breath in their schools. 3. Offer CATCH My Breath to small groups of students rather than in large groups. 4. Consider strategies for reaching priority populations.	Paola Litton – Lawrence County Health Department Dr. Jennifer Richards – Lawrence County Health Department	Schoolyears 2024–2025, 2025–2026, 2026–2027	
1.2 Advocate for schools to link students to tobacco cessation programs instead of punitive measures only. (*Policy recommendation, PHAB 5.2.2.A, 1c.)	Youth grades 9–12	Baseline: 6% (OHYES Tobacco Use Question) Target: 3% (OHYES Tobacco Use Question)	1. Present INDEPTH curriculum at superintendents meeting. 2. Arrange meetings with principals to explore implementing policy linking tobacco users to tobacco cessation programs instead of punitive measures only. 3. Recruit teachers and other school personnel to be trained in INDEPTH and serve as mentors/facilitators. 4. Continue to expand the program to more schools.	Paola Litton – Lawrence County Health Department Dr. Jennifer Richards – Lawrence County Health Department	Schoolyears 2024–2025, 2025–2026, 2026–2027	

1.3 Expand opportunities for social engagement for middle and high school youth	Youth grades 6–12	Aligned with SHIP MHA1. Youth Depression: Reduce the percentage of youth, ages 12–17, who experienced a major depressive episode within the past year Baseline: 41% report feeling sad or hopeless almost everyday for 2 weeks or more in a row during the past year (OHYES) Target: 38% (OHYES)	1. Reflect on “Party in the Park” offerings and plan to add activities that would appeal to middle and high school students, e.g. sports, music, games, etc. 2. Expand “Party in the Park” concept to a winter version, including activities that would appeal to middle and high school students. 3. Reach out to other agencies and schools to increase involvement.	Impact Prevention Lawrence County Health Department Potential partner – Third and Center	August 1, 2024–August 1, 2027	
1.4 Implement a suicide prevention campaign for at-risk youth.	All youth (especially males)	Aligned with SHIP MHA3. Youth Suicide Deaths: Reduce number of deaths due to suicide for youth, ages 8–17, especially residents of Appalachian counties Baseline: 3 (2023) Target: 0 Baseline: 15% seriously considered attempting suicide during the past year (OHYES) Target: 13% seriously considered attempting suicide during the past year (OHYES)	1. Work with youth-led “Sources of Strength” groups to create a new prevention campaign to be relevant and current to youth needs. 2. Review existing resources to identify emerging campaigns through ODH, OHMAS, Ohio Suicide Prevention. 3. Reconvene Youth Mental Health and Addiction to group to discuss recruitment strategies and plans.	Lawrence County Health Department and ADAMHS Board Partners: Faith-Based Community Members Impact Prevention Third and Center	January 1, 2025–May 1, 2026	

PRIORITY 2: Adult Mental Health and Addiction

Mental health is one of the top issues impacting the health of Lawrence County adults. One in five residents report experiencing 14 or more days of poor mental health per month, and the suicide death rate per 100,000 people is seven points higher for Lawrence County than for the state (21.9 compared to 14.8). Research indicates that populations at high risk for poor mental health include: **women; people experiencing homelessness; low-income individuals; individuals with a disability; lesbian, gay, and bisexual individuals; and veterans**. Although women are more likely to experience depression, men are much more likely to attempt suicide, especially veterans.

Drug overdose deaths are the leading cause of injury death in the United States. In Lawrence County, the death rate due to drug overdose per 100,000 people is 89.2, more than double the state death rate of 43.6 per 100,000. The majority of deaths due to pharmaceutical overdose involve prescription painkillers. Those who die from drug overdose are more likely to be **male, between the ages of 45 and 49, or Caucasian**, though the death rate among non-Hispanic Black men has been increasing. Research suggests that individuals who are **disabled, unemployed, or retired** are at higher risk.

Adverse childhood experiences (ACEs)—such as growing up in a home with violence, abuse, or mental health or substance abuse problems—are known to be a significant contributing factor. Those with six or more ACEs have significantly increased rates of depression and suicide; individuals with five or more ACEs are seven to 10 times more likely to report illicit drug use and addiction. Mental health trauma and substance abuse patterns are often transmitted across generations.

GOAL: Destigmatize mental health and addiction.

ALIGNMENT WITH NATIONAL PRIORITIES: Healthy People 2030: Improve Mental Health; Healthy People 2030: Reduce Drug and Alcohol Addiction

ALIGNMENT WITH OHIO'S STATE HEALTH IMPROVEMENT PLAN (SHIP): Priority Health Outcome: Improve Mental Health and Addiction

OBJECTIVE, STRATEGY	PRIORITY POPULATIONS	MEASURES	ACTION STEPS	LEAD CONTACT	TIMEFRAME	STATUS
2.1 Implement a "behavioral health is health care" campaign.	Adults (specifically males, farmers)	SHIP MHA4. Adult Suicide Deaths: Reduce the number of deaths due to suicide for residents of Appalachian counties (especially males)	1. Review existing resources to identify state campaigns through ODH, OHMAS, Ohio Suicide Prevention. 2. Identify campaigns to implement. 3. Reconvene Adult Mental Health and Addiction workgroup to implement a mental health stigma campaign with consideration to priority populations. 4. Look for funding opportunities to support ongoing campaign.	Lawrence County Health Department	August 1, 2024–September 1, 2025	
2.2 Continue implementing a "behavioral health is health care" campaign.	Adults (specifically males, farmers)	Baseline: 21.9 per 100,000 Target: 18.9 per 100,000	1. Continue "behavioral health is health care campaign," with expansions to additional priority populations as warranted.	Lawrence County Health Department	2025–2027	
2.3 Implement campaigns in which people in recovery from mental health and/or addiction tell their stories.	All adults	SHIP MHA7. Unintentional Drug Overdose Deaths: Reduce the number of deaths due to unintentional drug overdose for residents of Appalachian counties, especially among the following people: adults ages 25–34, adults ages 35–44, adults ages 45–54, males Baseline: 89.2 per 100,000 Target: 86.2 per 100,000	1. Examine the "recovery is beautiful" campaign from the Allen/Auglaize/Hardin ADAMHs board and consider its replication/adaptability for Lawrence County. 2. Reconvene Adult Mental Health and Addiction workgroup to discuss recruitment strategies and plans. 3. Contact community action/probation drug court/provider agencies/churches to identify and recruit persons in recovery to share their stories via video. 4. Work with Ohio University Southern, Marshall and/or other filmmakers to produce the videos. 5. Work with local TV stations i.e. WSAZ, WOWK, WCHS to promote the video(s). 6. Consider strategies for promoting campaign to priority populations. 7. Share videos with local legislators.	Lawrence County Health Department and ADAMHS Board	January 1, 2025–May 1, 2026	

PRIORITY 3: Adult Obesity

Ohio ranks seventh in the nation for adult obesity prevalence. In Lawrence County, 47 percent of adults are overweight or obese compared to 36 percent of Ohioans. Similarly, residents have high rates of physical inactivity (33%) and consume fewer fruits and vegetables than recommended. Though obesity impacts much of the population, **persons with disabilities, Black adults, and those living in rural areas are at higher risk**. Rural areas tend to have farther distances between residences and supermarkets, clinical settings, and recreational opportunities, a factor that likely impacts the ability to practice healthy behaviors that prevent obesity.

Adverse childhood experiences (ACEs) are known to be a significant contributing factor. Research indicates that individuals with one or more ACEs are 1.5 times more likely to become obese, and individuals of any ethnicity with four or more ACEs are 2.16 times more likely to become severely obese.

GOAL: Reduce overweight and obesity in Lawrence County by promoting healthy eating.

ALIGNMENT WITH NATIONAL PRIORITIES: Healthy People 2030: Improve health by promoting healthy eating and making nutritious foods available; Healthy People 2030: Reduce overweight and obesity by helping people eat healthy.

ALIGNMENT WITH OHIO'S STATE HEALTH IMPROVEMENT PLAN (SHIP): Health Behaviors: Improve Nutrition

OBJECTIVE, STRATEGY	PRIORITY POPULATIONS	MEASURES	ACTION STEPS	LEAD CONTACT	TIMEFRAME	STATUS
3.1. Implement Ohio's Good Food Here guidelines throughout Lawrence County. <i>(*Policy recommendation aimed at alleviating causes of health inequities, PHAB 5.2.2.A, 1c.)</i>	All adults (specifically persons with disabilities, rural residents)	Aligned with Healthy People 2030: Reduce the proportion of adults overweight or obese. Baseline: 47% (County Health Rankings, 2024) Target: 44% Increase the number of local organizations adopting Ohio Food Service guidelines. Target: One gas station or convenience store using these guidelines in 50% of the villages or townships.	1. Meet with the county developmental disabilities board to discuss these guidelines to identify facilities in which to implement. 2. Work to get at least one gas station or convenience store in 50% of the villages or townships to implement the guidelines. 3. Continue to address places reaching vulnerable populations in the county i.e. veterans, low-income people.	Healthy Opportunities Coalition – Zoie Clay	August 1, 2024–August 1, 2027	
3.2. Implement a produce prescription program.	Low-middle class populations, individuals with chronic diseases and those 65+	Aligned with Healthy People 2030: Increase fruit consumption by people aged 2 and older. Baseline: 50% consumed fruit less than one time per day (ODH BRFSS: 2019 Annual Report, Regional Data) Target: 45% Aligned with Healthy People 2030: Increase vegetable consumption by people aged 2 and older. Baseline: 21.2% consumed vegetables less than one time per day (ODH BRFSS: 2019 Annual Report, Regional Data) Target: 18%	1. Examine other counties' programs to identify potential funders. 2. Target suppliers to become a program partner and to accept the produce prescription vouchers. 3. Educate medical and behavioral health care providers about the program and encourage them to write the prescriptions. 4. Educate recipients about program benefits and general nutrition knowledge.	Healthy Opportunities Coalition – Zoie Clay Lawrence County Health Department – Dr. Jennifer Richards	January 1, 2025–August 1, 2027	

MONITORING AND EVALUATING THE CHIP

Lawrence County Health Department will follow the Plan-Do-Study-Act quality improvement model throughout the implementation of the CHIP. LCHD will conduct ongoing monitoring of the implementation of the strategies and short-term outcomes to ensure they are being applied for the best possible outcome. LCHD will convene meetings of the Priority Issue Groups as indicated in the Action Plans and will update the status of the Action Plans at least once per year. LCHD in collaboration with the CHIP Partners will: (1) Study the results of the strategies implemented to determine whether the strategy was effective and what factors contributed to the degree of success and (2) Act on those results as warranted. LCHD will make revisions to the CHIP as needed to maximize the efforts to achieve the vision of “a society in which all people can achieve their full potential for health and well-being across the lifespan through making the healthier choice the easier choice and overcoming barriers to economic vitality.”

Contact Us

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⁴⁰PHAB, “What’s in a Method? Quality Improvement Methodologies: Determining the Best Approach for Incorporating QI into Your Agency’s Practice,” 2020, <https://phaboard.org/wp-content/uploads/ComparisonFrameworkFeb2020.pdf>.

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APPENDIX A: CHIP PARTNERS’ PRIORITIZATION OF TOP HEALTH ISSUES

The CHIP Partners completed a confidential ranking via polling software of the top health issues for both adults and youth. The votes were compiled by weighting each ranking of number one as three points, each ranking of number two as two points, and each ranking of number three as one point.

The number of votes for each health issue based on rankings by CHIP Partners identified Youth Mental Health and Addiction, Adult Mental Health and Addiction, and Adult Physical Activity, Nutrition, and Weight Status as the top issues for the CHIP.

